# 7.3 Acute Glomerulonephritis

| Assessment | Nursing Diagnosis | Plan / Expected Outcome | Nursing Interventions | Evaluation |
|---|---|---|---|---|
| **Subjective**<br>• "My face is puffy in the morning and my urine looks like cola."<br>**Objective**<br>• Puffy around the eyes<br>• Blood and protein in the urine<br>• BP 160/100 mmHg<br>• Passing little urine | **ACTUAL**<br>**Excess fluid volume** related to decreased glomerular filtration as evidenced by edema, oliguria and hypertension. | Within 1 week, less edema and BP on target. | • Weigh every day.<br>• Strict I&O chart.<br>• Limit sodium and fluid as prescribed.<br>• Give antihypertensives and diuretics as prescribed.<br>• Follow urea, creatinine and potassium. | Edema less, BP controlled: **goal met**. |
| **Subjective**<br>• "I feel tired."<br>**Objective**<br>• Tired<br>• Doing less than usual | **ACTUAL**<br>**Fatigue** related to disease process as evidenced by tiredness. | Within 1 week, says he has more energy. | • Encourage rest in the acute phase.<br>• As the edema goes down, build up activity slowly.<br>• Plan care so he can rest. | More energy: **goal met**. |
| **Subjective**<br>• *No specific complaint*<br>**Objective**<br>• Hypertensive<br>• Fluid overloaded | **RISK**<br>**Risk for ineffective cerebral tissue perfusion** as evidenced by risk factors: severe hypertension. | No neurological complications. | • BP every 4 hours.<br>• Report headache, changes in vision or seizures.<br>• Give antihypertensives as prescribed. | No complication: **goal met**. |